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The Minister of VWS

P.O. Box 20350

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Date

October 17, 2023

Our reference

z2023-00615

Your letter dated

January 27, 2023

Contact person

Your reference

3503495-1042729-DICIO

Subject

Amendment Act Data Processing VWS II

Dear Mr. Kuipers,

In your letter dated January 27, 2023, the Authority for Personal Data (AP) was consulted based on the provisions of Article 36, fourth paragraph, of the General Data Protection Regulation (GDPR), regarding the draft amendment of several laws in the area of the Ministry of Health, Welfare and Sport to strengthen the legal basis for data processing (hereinafter: the draft). The request for advice was supplemented on March 2, 2023.

Scope of the draft

The draft concerns a consolidation act consisting of thirteen separate parts aimed at amending various laws in the healthcare sector.¹ The draft assigns and amends powers and obligations regarding the processing of personal data, including health data, for various actors within and outside the healthcare sector. The AP observes that this again concerns the resolution of a few specific situations, but that a more overarching and coherent approach to the problems surrounding the legal bases for the processing of health data is still awaited. Bottlenecks in the processing of health data for various purposes (such as healthcare provision, but also quality monitoring, collaboration, policy and planning, scientific research) have already been signaled and recognized multiple times from various perspectives. In this regard, the AP advises you to proceed with the necessary urgency in the project you have initiated regarding the 'reassessment of legal bases'² to achieve more clarifications and concrete results.

¹ The numbering of the parts aligns with the numbering in Chapter 2 of the explanatory memorandum accompanying the draft.

² Parliamentary Document 27 529, no. 276.

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The AP has examined whether the draft regarding the processing of personal data is in accordance with the Charter of Fundamental Rights of the European Union (EU), the general legal principles of the EU, the GDPR, and other relevant law, whereby it must be sufficiently demonstrated that the principle of proportionality is met, by:

1. the draft being suitable to achieve the pursued objective of general interest or the protection of the rights and freedoms of others (suitability);
2. the objective cannot reasonably be achieved as effectively by other means that are less intrusive to the fundamental rights of the data subjects (subsidiarity);
3. the interference is not disproportionate to that objective, which particularly implies a weighing of the importance of the objective and the seriousness of the interference (proportionality);
4. the draft is sufficiently clear and precise regarding the scope and is predictable in its application (legal certainty);
5. the draft sufficiently indicates under which circumstances and conditions personal data may be processed, thereby ensuring that the interference is limited to what is strictly necessary (substantive and procedural safeguards); and
6. the draft is binding under national law (bindingness under national law).

The AP has objections to the draft on the following points and advises against continuing the procedure unless the objections are resolved.

- The proportionality of a legal obligation for healthcare providers to share data, including health data, of uninsured patients with municipalities for the purpose of providing outreach care has not been sufficiently demonstrated (part 7).
- The proportionality of a legal obligation for healthcare providers to share data, including health data, of all patients who have previously participated in a population study or screening with the executor of that study or the RIVM has not been sufficiently demonstrated (part 9).
- The explanatory memorandum lacks a detailed assessment against Articles 9 and 10 of the GDPR in the context of the provision of special personal data for the preparation or execution of a judicial authorization for involuntary admission and stay in a Wzd care institution (part 11).

In addition, the draft gives the AP reason to make comments regarding the following parts:

- National Contact Point for EHealth (part 1)
- Contact persons CIJZ (part 3)
- Overlap Wmo 2015, Youth Act, and Wlz (part 4)
- Supervision IGJ on (parts of) Wabv pz (part 13)

Assessment of the personal data aspects of the draft does not give the AP reason to make comments regarding the following parts:

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- Use of BSN representatives and authorized persons for access to PGO (part 2)
- Supervisory committees (part 5)
- Retention period for Wmo clients' data (part 8)
- Data exchange JGZ-RIVM (part 10)
- Provision of policy information to the Minister of VWS (part 12)

No advice is needed regarding part 6 (double setup requirement Wmo 2015) as it does not relate to the processing of personal data.

Advice

1. National Contact Point for eHealth

Within MyHealth@EU3, a European digital infrastructure consisting of a network of national contact points for eHealth (NCPeH), each member state can establish a national NCPeH that connects with other European contact points.⁴ An NCPeH is a national hub that serves as a communication gateway between the healthcare information systems of EU member states, thereby making healthcare data

digitally available and receiving it for the provision of care.^{5 6}

Article 15k of the Act on Additional Provisions for the Processing of Personal Data in Healthcare (hereinafter Wabvpz) designates the Minister of VWS as the Dutch NCPeH. The Minister is authorized to receive healthcare data of patients from other EU member states from Dutch healthcare providers and to provide that data to the NCPeH of the relevant member state.^{7 8} Additionally, the Minister forwards the healthcare data originating from another member state to the Dutch healthcare provider that requests it.⁹

The legislative proposal goes further than it seems

The establishment of an NCPeH effectively means participation in the European exchange of personal data.¹⁰ This exchange will be further expanded when the EU proposal for a regulation on a European Health Data Space (EHDS)¹¹ is adopted.¹²

This is a far-reaching step, which is also being critically monitored by the House of Representatives.¹³ Not only is the structural exchange of patient data "across borders" a novelty, but similar issues arise as those previously encountered with the (Dutch) National Switch Point, such as the delineation of access to patient data (how is it prevented that data is requested from a person who is not receiving treatment from the requesting healthcare provider?).¹⁴ The National Switch Point, as is known, had a turbulent history.¹⁵

Another point that may be sensitive – and has no precedent – is the role of the government in these processes. In the case of the Dutch National Switch Point, it was specifically chosen to have the sector itself¹⁶ be the data controller.

The explanatory memorandum does not address these more fundamental issues.

The directive for regulation 6.4 states the following regarding the admissibility of omnibus laws:

"An amendment law that regulates multiple substantive topics will only be pursued in principle if:

- a. the different components are interconnected;
- b. the different components are not of a scope and complexity that justifies a separate legislative proposal; and
- c. it is not expected in advance that one of the components is so politically controversial that a proper parliamentary treatment of the other components is jeopardized."

Given the above, it is possible that this component of the draft does not meet condition c.

The AP advises that the explanatory memorandum pay more attention to the position of the draft in relation to the EHDS, the described history, the role of the government, and the introduction of structural exchange of patient data across borders. The AP also advises considering whether this component should not be regulated in a separate legislative proposal. The explanatory memorandum should then be expanded in such a way that a well-founded assessment can be made regarding the intended European exchange of patient data.

¹¹ COM(2022)197 (presented on May 3, 2022).

¹² So also KNMG: "The KNMG interprets this proposal as a preparation for the implementation of the future EHDS regulation in the area of the primary use of patient data for the provision of care within the borders of the EU" (response to the internet consultation, February 14, 2023, p. 2).

¹³ Parliamentary documents 36 121.

¹⁴ See also the advice of the Data Protection Authority of June 14, 2007 (z2007-577), appendix p. 5-6.

¹⁵ Electronic patient record - Wikipedia.

¹⁶ Namely, the Association of Healthcare Providers for Healthcare Communication (VZVZ; Home | VZVZ).

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Processing in the context of management

Article 15k, second paragraph, under b and c of the Wabvpz regulates the European data exchange between healthcare providers via the Dutch NCPeH. Article 15k, fifth paragraph of the Wabvpz, in connection with article 15k, first paragraph, Wabvpz, further stipulates that the Minister of VWS processes personal data - including health data - for the purpose of managing the "contact point for e-health."¹⁷

The processing pursuant to article 15k, second paragraph, under b and c of the Wabvpz can be qualified as respectively "requesting" and "providing by means of forwarding."¹⁸ It is unclear which additional processing is provided for in the context of managing the contact point for e-health.

The AP advises clarifying in the explanatory memorandum which processing is provided for in the context of managing the contact point for e-health and to substantiate the necessity of processing health data in this regard.

Data to be processed

The draft does not indicate the dataset that will be exchanged.¹⁹ The explanatory memorandum mentions a "patient summary." It also appears from the explanatory memorandum that the content of this "patient summary" is determined at the European level.²⁰ However, there is no specific reference to European legislation in this regard.

By leaving the content of the term "patient summary" entirely open and not specifically referring to European legislation in this regard, the draft is insufficiently specific on this point.

The AP advises defining the dataset to be exchanged.

Legal basis for processing

The draft is unclear about the precise legal bases²¹ for the distinct processing activities. Consent and legal basis are used interchangeably in the explanatory memorandum without it being clear which basis applies to which processing.

The AP advises carefully distinguishing the various processing activities within the proposed European data exchange in the explanatory memorandum and clearly indicating the legal basis (article 6 GDPR) or exception grounds (article 9 GDPR) for each processing activity. It should also be indicated how the processing relates to article 7:457 BW and article 15a Wabvpz.

17 "E-health" is (broadly) defined in article 1 Wabvpz. "Contact point for e-health" is not defined as such in the draft, but given the context, it must be assumed that this refers to the NCPeH.

18 Article 4 under 2 GDPR.

19 The draft only refers to "personal data, including health data" (article 15k, second paragraph, under b and c, and fifth paragraph Wabvpz).

20 Explanatory memorandum p. 2.

21 Or exception grounds in article 9, second paragraph, GDPR.

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3. Contact Persons CIZ

Article 3.2.3, first paragraph, of the Long-Term Care Act (Wlz) assigns the Center for Indication of Care (CIZ) the task of issuing an indication decision upon request from an insured person, in which the right to care for the insured person is established. According to the explanatory memorandum, there are cases where competent clients prefer that someone else acts as a contact person for the purpose of an application.

In Article 9.1.1a, first and second paragraphs, of the Wlz, grounds are included for the CIZ to process the data about the contact person and to share this data with the care office. The third paragraph also provides a basis for the care office to discuss data about (the health of) the client with the contact person, if necessary, to determine which care provider can offer appropriate care. With the proposed Article 9.1.1a, fourth paragraph, of the Wlz, the processing in Article 9.1.1a, first and third paragraphs, of the Wlz is exempted from the prohibition in Article 9, second paragraph, part h, of the General Data Protection Regulation (GDPR).

In the opinion of the AP (Dutch Data Protection Authority), it is not necessary to include a separate basis for the processing of the contact person's data. Everyone can be assisted in dealings with administrative bodies or be represented by an authorized representative in the interest of their affairs, see Article 2:1, first paragraph, of the General Administrative Law Act (Awb). This also applies to a client who designates someone to act as a contact person on their behalf for the purpose of an application for an indication decision. The AP is of the opinion that this designation sufficiently addresses the processing of contact details of that contact person, that these details are shared with the care office to find an appropriate care provider, and that medical data of the client are discussed with the contact person.

The AP advises that the proposed provision be deleted in its entirety.

****4. Overlap Wmo 2015, Youth Act, and Wlz****

The Wlz grants the CIZ the authority to issue an indication decision that establishes the right to care of a Wlz-insured person.²² The Wmo 2015 and the Youth Act grant the municipality the authority to provide a (customized) provision or personal budget to a Wmo client or a youth.²³ If a Wmo client or a youth is eligible for care under the Wlz, the municipality may refrain from providing a provision under the Wmo or the Youth Act.²⁴

²² Article 3.2.3, first paragraph, Wlz. To assert their right to care in kind, an insured person must approach a care provider that has an agreement with the Wlz executor, often the care office, based on Article 3.3.1, second paragraph, Wlz.

²³ Article 2.3.1 Wmo 2015, Article 2.3.1 Wmo 2015, Article 2.3 Youth Act, Article 8.1.1, first paragraph, Youth Act.

²⁴ Article 2.3.5 Wmo 2015 and Article 1.2, first paragraph, part c, Youth Act. With the Wlz-register assessment product from the Information Bureau (IB), the municipality can check in real-time whether a citizen applying for a Wmo customized provision also has a Wlz indication from the CIZ. With the Wlz signals product from the IB, the municipality can receive a daily file with overlap signals via the IB.

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According to the explanatory memorandum, there is currently no basis in the Youth Act for the exchange of information between the CIZ and the municipality without the client's consent. As a result, municipalities cannot adequately ascertain whether and when a client receives a provision based on the Wlz and which provision this concerns.

It is proposed to amend the Youth Act and the Wmo 2015 in such a way that a municipality receives information directly and without the intervention of the client from the CIZ and the care office. This concerns whether a Wlz indication has been issued and the form of delivery of care and the start and end dates thereof.

The proposed Article 7.4.0, fourth paragraph, of the Youth Act provides a basis for the CIZ to inform the municipality in the case of a granting indication decision, namely when an "indication decision has been issued establishing that a youth is designated for care [...]". However, the explanatory memorandum seems to assume that a notification to the municipality occurs in the case of both a

granting indication decision and a rejecting indication decision.

The AP advises removing this contradiction by correcting the explanatory memorandum on this point and stating that a notification may only be made in the case of a granting indication decision.

7. Uninsured

When a care provider provides medically necessary care to a wrongly uninsured person, these costs are not reimbursed by a health insurer. If a wrongly uninsured person does not have coverage under the Wmo and Wlz, the municipality provides a file of all Wmo clients, which is matched by the IB with files of clients who have a Wlz indication from the CIZ.

25 Explanatory memorandum p. 6.

26 Article 7.4.0, fourth paragraph, Youth Act. The draft provision corresponds to the current Article 5.2.5, third paragraph, Wmo 2015, in which the possibility of issuing a Wlz signal by the CIZ to the municipality already exists.

27 Article 7.4.0, fifth paragraph, Youth Act and Article 5.2.5, fourth paragraph Wmo 2015.

28 Chapter 3, paragraph 3 Wlz; According to the explanatory memorandum, paragraph 2.4.1, this concerns the following forms of delivery:

- Living at home (full package at home (VPT), modular package at home (MPT), personal budget (PGB))
- Part-time stay
- Stay in an institution

29 Explanatory memorandum p. 6.

30 In the proposed Article 5.2.5a, second paragraph, Wmo 2015 explained as care or other services as referred to in Article 11 Zvw, excluding forms of care or services excluded by or under that law, and only insofar as the care provider considers the provision of such care to be medically necessary, given the nature of the care.

31 A wrongly uninsured person is an individual who is not insured under the Health Insurance Act (Zvw) but is required to be. The group of those required to be insured is essentially the same as the group of insured individuals under the Long-Term Care Act (Wlz) and consists of residents and non-residents who perform work in the Netherlands that is subject to wage tax. Military personnel on active duty and conscientious objectors are not required to be insured under the Zvw. Foreigners who do not legally reside in the Netherlands are not insured under the Wlz and therefore are also not subject to insurance obligation under the Zvw (they do, however, have the right to care, see Article 122a Zvw). See Article 1, under e, Zvw, Article 2, first and second paragraphs, Zvw and Article Date

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Regarding sufficient means to pay the bill themselves, the care provider can apply for a subsidy based on Article 6 of the

Subsidy Regulation for Medically Necessary Care for the Uninsured (SOV)³².

According to the explanatory memorandum, not having health insurance is almost always a signal that people have even more (socio-economic) problems.³³ According to the explanatory memorandum, the municipality has a duty under the Wmo 2015 to reach and assist vulnerable individuals who do not seek help on their own, with the aim of accepting necessary help and support.^{34 35}

If the help is accepted, the municipality assesses based on Article 2.3.2 Wmo 2015 which help and support is necessary.

Additionally, follow-up assistance according to the explanatory memorandum may consist of social support, (labor) participation, debt assistance, and/or guiding the uninsured to a health insurance.

With the proposed Article 5.2.5a, first paragraph, Wmo 2015, a legal obligation is introduced for care

providers who have provided medically necessary care to an unjustly uninsured person or to someone whose insurance cannot be determined, to provide personal data, including health data and the BSN, about the patient to the municipality or the GGD.^{36 37} The data will only be used to approach the insured and to provide intrusive care. If that help is not accepted, the municipality withdraws.³⁸

In the proposed Article 5.2.5a, fifth paragraph, Wmo 2015, it states that the legal obligation in Article 5.2.5a, first paragraph, Wmo 2015, breaks the confidentiality obligation of care providers³⁹. The interest in passing on the data of the uninsured to the municipality and/or the GGD is considered to outweigh the medical professional secrecy, as the report is seen as crucial to providing help to the uninsured.

2.1.1, first paragraph Wlz. The circle of insured persons for the Wlz can be expanded or restricted by or pursuant to an AMvB, for which purpose the Decree on the Expansion and Restriction of the Circle of Insured Persons for National Insurance 1999 applies.

32 Entered into force on March 1, 2017.

33 The explanatory memorandum lists (threatening) problems in the areas of work and income, health, care, debts, safety, housing, and education as examples.

34 Although the provision of intrusive care is not explicitly included in the Wmo 2015 at this time, according to the explanatory memorandum, it appears from the parliamentary history that public mental health care, including intrusive care, is a common task. The Bill for Addressing Multiple Problems in the Social Domain (Wams, parliamentary document 36295, no. 2) proposes to explicitly include public mental health care in the Wmo 2015.

35 Follow-up assistance can, according to p. 13 of the explanatory memorandum, consist of social support, (labor) participation, debt assistance, and guiding the uninsured to a health insurance.

36 The processing is based on the grounds of Article 6, part c, GDPR and Article 9, second paragraph, part h, GDPR.

37 With the proposed Article 5.2.5a, third paragraph, Wmo 2015, care providers are also authorized to provide the data to GGD's, which then provide the data to the municipality. With the proposed Article 5.2.5a, fourth paragraph, Wmo 2015, GGD's and municipalities are authorized to process the received data. According to p. 15 of the explanatory memorandum, most municipalities have outsourced the tasks associated with follow-up assistance to regional GGD's. According to p. 53 of the explanatory memorandum, municipalities and GGD's are free to make arrangements about this among themselves.

38 Explanatory memorandum p. 37.

39 Article 7:457, first paragraph, Civil Code and Article 88 BIG Act.

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****Proportionality****

Firstly, the AP considers it insufficiently plausible that the legal obligation is suitable to achieve the desired goal, as it is unclear how the municipality can reach a homeless or roofless person after receiving a report. This is while, according to the explanatory memorandum, homeless and roofless individuals constitute a large part of the target group.⁴⁰

Secondly, the necessity of the legal obligation is also based on the task that the municipality would have to encourage wrongly uninsured individuals to insure themselves. However, under Articles 9a-9d of the Health Insurance Act (Zvw), the CAK can already take various measures aimed at insuring wrongly uninsured individuals.⁴¹ Moreover, the CAK knows exactly which individuals are wrongly uninsured through file comparison.⁴² Moreover, Article 7b.1, third paragraph, of the Health Insurance Regulation already contains a basis for the CAK to provide data about wrongly uninsured individuals

against whom measures have been taken to the municipality. The explanatory memorandum does not clarify why, in addition to the existing possibilities for the CAK, healthcare providers must now also provide data to the municipality.

Thirdly, the AP finds it insufficiently plausible that the wrongful lack of health insurance should be linked in this way to, for example, participation in a debt assistance program or (labor) participation. The processing of the fact that medically necessary care has been provided is thus linked to purposes that, in principle, do not have much to do with that fact.

In light of all these objections, the AP advises against continuing this draft if these issues regarding proportionality cannot be resolved. The AP recommends that the draft be amended so that the municipality learns which individuals are wrongly uninsured not through the healthcare provider, but through the CAK. A basis must first be created that meets the requirements of proportionality, with the aim of enabling the municipality to offer (social) assistance.

****Data to be processed and retention period****

With this draft, healthcare providers are obliged to provide personal data, including health data and the BSN, to the municipality to the extent necessary to assess whether it is necessary to provide assistance.

40 Explanatory memorandum, p. 15.

41 The CAK first sends a written reminder to the wrongly uninsured individuals to insure themselves within three months (Article 9a Zvw). If the insured party is not insured within three months, as evidenced by file comparison, the CAK imposes an administrative fine amounting to three times the standard monthly premium (Article 9a, first paragraph, and 9b Zvw). After that, another administrative fine can be imposed, accompanied by an order to insure themselves within three months (Article 9c Zvw). If the insured party is still not insured, the CAK will, on their behalf, automatically conclude a health insurance policy in which they are insured (Article 9c Zvw).

42 Article 9a, first paragraph, Zvw; The file of military personnel in actual service, the file of conscientious objectors, and the file of Zvw-insured individuals are deducted from the file of Wlz-insured individuals, resulting in a file of wrongly uninsured individuals, as indicated in Cremers, 'T&C; Health Law, commentary on Article 9a Zvw', paragraph 2.3, Article 9a.

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the provision of public mental health care is designated. According to the explanatory memorandum, it concerns at least the fact that a care provider has provided uninsured Zvw care.⁴³

Firstly, the proposed Article 5.2.5a, first paragraph, Wmo 2015 lacks a description of which data must be provided, and secondly, it is unclear whether, in addition to the fact that Zvw care has been provided, other personal data must also be provided. As a result, the provision is insufficiently predictable for patients, and there is a risk that more personal data will be provided than necessary.⁴⁴

The AP advises to specify in the legislative text of the draft which data must be provided by the care provider to the municipality.

The AP also advises to include in the draft that any obtained data will be deleted if the offered assistance is not accepted.

Establishment of insurance obligation

It is unclear how a care provider can determine whether someone is or is not subject to the insurance obligation in the sense of the Zvw. Inquiry by the AP shows that care providers can only find this out through information provided by the patient themselves. This makes it possible for unnecessary data to be provided to the municipality even for individuals who are not eligible for health insurance in the sense of the Zvw. The explanatory memorandum acknowledges that this is undesirable, but no further

solution is offered.⁴⁵ Especially concerning foreigners who do not legally reside in the Netherlands, there is a risk that they will avoid care, knowing that their data will be shared with the municipality. The explanatory memorandum states that in more than half of the care claims submitted under the SOV, it concerns non-Dutch individuals who are often not subject to the insurance obligation.⁴⁶

The AP advises to explain in the explanatory memorandum how care providers can determine whether a patient is or is not subject to the insurance obligation for the Zvw.

Assessment space for reporting

It is also unclear how care providers are expected to weigh whether a report is necessary "to assess whether the provision of public mental health care is indicated" (proposed Article 5.2.5a, first paragraph). From the explanatory memorandum and based on inquiries with the ministry, it appears that it is provided that care providers must report to the municipality each time they invoke the SOV. In fact, there is no assessment space for care providers to weigh when a report is necessary.

43 Explanatory memorandum p. 36.

44 Consideration 41 GDPR.

45 Explanatory memorandum p. 15.

46 Explanatory memorandum p. 15.

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The AP advises that the explanatory memorandum should explain how healthcare providers are expected to make this assessment. Since it is intended that healthcare providers may assess for themselves when to make a report, the AP also advises considering whether a legal authority is more appropriate than a legal obligation.

9. Data Exchange RIVM Prevention Programs

The RIVM is tasked with ensuring the management and coordination of the execution, registration, monitoring, and evaluation of population screenings designated by general administrative order.⁴⁷ Additionally, the RIVM is responsible for the national steering and guidance of prevention programs established by general administrative order.⁴⁸

If the result of a population screening or screening is positive, the patient is referred to a healthcare provider or laboratory for follow-up diagnostics or treatment. According to the explanatory memorandum, it is necessary for the quality of a prevention program that the RIVM has access to the (follow-up) diagnostic data of individuals who have been referred after a positive result, or in whom an anomaly has been detected after a negative result. These feedbacks from healthcare providers and laboratories to the RIVM are currently based on the legal tasks of the RIVM. However, according to the explanatory memorandum, healthcare providers have become more reserved in this regard since the introduction of the GDPR.⁴⁹

This draft introduces legal obligations for healthcare providers to provide (follow-up) diagnostic data about their patients to the executors⁵⁰ of prevention programs, and for executors to provide that data to the RIVM.⁵¹ Additionally, a legal basis is provided for executors to process the data and the BSN so that they can link the (follow-up) diagnostic data to the data available to them.⁵²

Scope of Obligation

The AP objects to the scope of the legal obligation for healthcare providers to share certain health data with the executor of a population screening or screening. The situation where withholding consent by the individual concerned could lead to a distorted view of the effectiveness of a prevention program will only occur in a limited number of rare conditions. After all, few participants are referred and diagnosed for those conditions. It is advisable to limit the legal obligation to that limited scope.

47 Article 12a Public Health Act.

48 Article 3, first paragraph, part c, RIVM Act; These are the population screenings for breast cancer, cervical cancer, bowel cancer, the pre- and neonatal screenings, and the national programs for flu prevention and pneumococcal vaccination for adults, as stated in Article 1, second paragraph, RIVM Decision.

49 Explanatory memorandum p. 18.

50 The executor can be the RIVM but can also be another organization, such as the Foundation for Population Screening Netherlands, Explanatory memorandum p. 19.

51 Proposed Article 7a, third paragraph, second sentence, RIVM Act and Article 7a, first and second paragraphs, RIVM Act.

52 Proposed Article 7a, third paragraph, first sentence, RIVM Act.

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Category: rare diseases. For the remainder, the processing can be based on the patient's consent.

The AP advises limiting the legal obligation to rare diseases where withholding consent could lead to a distorted view of the effectiveness of a prevention program.

Reason for feedback

The text of the law and the explanatory memorandum do not sufficiently clarify how a healthcare provider can know if their patient has previously participated in a prevention program and received a negative result. Inquiries by the AP reveal that it is anticipated that healthcare providers can determine whether a patient falls within the target group of the prevention programs based on the patient's age, gender, or specific health situation. Healthcare providers ask patients diagnosed with a condition that falls under a prevention program and also belongs to the target group whether they have participated in the relevant prevention program.

This approach does not enhance the reliability of monitoring and evaluating the effectiveness of the population screening. There is a possibility, for example, that healthcare providers are unaware of changes in the list of conditions tested in the prevention programs, that healthcare providers ask their patients in different ways whether they have participated in a prevention program, or that patients themselves do not clearly remember whether they have participated before. In light of this, it is questionable whether the objective underlying the legal obligation can indeed be achieved with the current form of this concept.

The AP advises reconsidering whether the legal obligation is suitable for achieving the intended goal in the case of a negative result. The AP recommends continuing the introduction of the provision only if healthcare providers are actually able to comply with that legal obligation.

Data to be processed

With this concept, healthcare providers are required to provide personal data of patients in the target group of a prevention program to the operator of a prevention program. However, there is a lack of a sufficiently clear and precise description of which data must be provided to the operator. As a result, the obligation is insufficiently predictable, and there is a risk that more personal data will be provided than necessary.⁵³

Operators of prevention programs, in turn, are required to provide the obtained (subsequent) diagnostic personal data to the RIVM. However, no further requirements are set regarding which data must be provided and how this should be done. According to the explanatory memorandum, it is intended that the operator of a prevention program, if it is not the RIVM itself,

53 Recital 41 GDPR.

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no health data at the individual level needs to be provided to the RIVM.⁵⁴ However, the text of the law does not exclude the possibility that such data may still be involved, even though the RIVM does not require that data.

The AP advises that the text of the law in the draft should specify which data must be provided by the healthcare provider to the executor of the prevention program. Additionally, the AP advises that the data to be provided by the executor to the RIVM should be limited to anonymized data.

11. Involuntary admission to Wzd care institution

Article 2.3, second paragraph, of the Forensic Care Act (Wfz) gives the criminal court the authority to issue a judicial authorization for involuntary admission and residence in a Wzd⁵⁵ care institution. The judge may issue that authorization if he believes that the criteria in Article 24 of the Wzd⁵⁶ are met.

The implementation of these authorizations is laid down in Articles 28a to 28c of the Wzd. Since the entry into force of the Wzd (2020), there have been placement issues, and executing parties often fail to find a suitable care place in a timely manner.⁵⁷

To find a place on time, it is important that the parties involved - Public Prosecution Service, Wlz executor⁵⁸, the municipality where the client resides, the health insurer, the CIZ, the care provider, and the Judicial Institutions Service - can exchange the necessary data early in the procedure of Article 2.3, second paragraph, Wfz, which is needed to find a suitable place.⁵⁹

The proposed Article 28a, sixth paragraph, Wzd makes this possible. The data may only be exchanged to the extent that it is necessary for the preparation or execution of a judicial authorization granted under Article 2.3, second paragraph, of the Wfz.

In the proposed Article 28a, sixth paragraph, Wzd, it is stated that health data⁶⁰ and data of a criminal nature⁶¹ will also be provided in connection with the preparation or execution of a judicial authorization for involuntary admission and residence in a Wzd care institution.

In the explanatory memorandum, a detailed assessment against Articles 9 and 10 of the GDPR is lacking in this regard.

The AP advises that the explanatory memorandum should clarify which considerations have been made in the context of Articles 9 and 10 of the GDPR.

13. Supervision by IGJ on (parts of) Wabvpz

To date, the supervision of the Act on Additional Provisions for the Processing of Personal Data in Healthcare (Wabvpz) by IGJ has been shaped in relation to the concept of "good care" from Article 2 of the Act on Quality, Complaints, and Disputes in Healthcare (Wkkgz). This means that IGJ can only take enforcement action for violations of the Wabvpz if this also results in a violation of Article 2 Wkkgz.⁶² For example, when inadequate information security leads to a deterioration in the quality of care provided.

In the new Chapter 3b of the Wabvpz, IGJ will receive the following enforcement instruments in the context of supervision of certain provisions of the Wabvpz:

- a. written instruction;
- b. written order; and
- c. penalty under coercive fine.

This concerns Articles 5 through 12, 15d, 15e, and 15j of the Wabvpz⁶³.

The draft also stipulates that IGJ and the AP will provide each other with the information and data

necessary for the performance of their duties, as far as reasonably necessary for the fulfillment of that duty. This does not include data from medical files.⁶⁴

60 As referred to in Article 9 of the GDPR. See specifically footnote 71: “diagnosis” and “Indication and care needs.”

61 As referred to in Article 10 of the GDPR. The fact that someone will have to be placed in an institution through a Wfz procedure is, according to the explanatory memorandum, a personal data of a criminal nature. This is the only personal data of a criminal nature that will be provided (Explanatory Memorandum p. 24).

62 “Good care,” the primary area of supervision for IGJ (Article 24 Wkkgz).

63 Articles 5-12 of the Wabvpz concern patient identification and use of BSN. Articles 15d and 15e of the Wabvpz address access to the patient file. Article 15j of the Wabvpz concerns (in particular) information security.

64 Article 16c of the Wabvpz.

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execution of a judicial authorization granted under Article 2.3, second paragraph, of the Wfz.

In the proposed Article 28a, sixth paragraph, Wzd, it is included that health data⁶⁰ and data of a criminal nature⁶¹ will also be provided in connection with the preparation or execution of a judicial authorization for involuntary admission and residence in a Wzd care institution.

In the explanatory memorandum, a detailed assessment against Articles 9 and 10 of the GDPR is lacking in this regard.

The AP advises that the explanatory memorandum should clarify which considerations have been made in the context of Articles 9 and 10 of the GDPR.

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In the draft, IGJ is granted enforcement powers regarding Articles 5 through 12, 15d, 15e, and 15j of the Wabvpz⁶⁵, even if – unlike now – there is no violation of Article 2 of the Wkkgz.⁶⁶

The background for this is:

“To effectively supervise, supervisory and enforcement options are necessary [...] The text of the Wabvpz does not explicitly prescribe which supervisory authority oversees the articles from the law that describe obligations for care providers [...] Due to the increasing importance of digitization and electronic data exchange, the importance of good information security in healthcare is also increasing. It is desirable that IGJ has the explicit authority to supervise this, even when there is no direct relationship to the quality of care provided [...] The policy objective is that the enforceability of the Wabvpz is adequately secured. This objective is achieved by explicitly regulating supervision and enforcement of the Wabvpz in the Wabvpz.”⁶⁷

The AP emphasizes that the Wabvpz does not need to explicitly prescribe which supervisory authority oversees the articles from the Wabvpz that describe obligations for care providers. The AP is responsible for both the supervision of the application of the GDPR and for the processing of personal data in national legislation such as the Wabvpz.⁶⁸ The IGJ can, insofar as “good care” is at stake or threatened, also – whether or not together with the AP⁶⁹ – supervise the Wabvpz. No explicit indication in the law is needed for this.

The draft creates a duplication in the supervisory field of the AP and the IGJ. Both are authorized to supervise without a relationship to the quality of care provided. The potential risks of this duplication can be mitigated by adjusting the Cooperation Protocol AP-IGJ. Nevertheless, the question remains whether the IGJ already has sufficient instruments to effectively supervise the provisions of the Wabvpz.

The AP advises that the explanatory memorandum should better substantiate why the current instruments of the AP and the IGJ are not sufficient to supervise the provisions of the Wabvpz. Should this lead to the conclusion that the enforceability of the Wabvpz is already sufficiently secured, the AP advises not to continue this part of the draft.

Workload AP

The AP notes that the draft is not expected to necessitate additional efforts by the AP.

65 Articles 5-12 of the Wabvpz concern patient identification and the use of BSN. Articles 15d and 15e of the Wabvpz address access to the patient file. Article 15j of the Wabvpz pertains (in particular) to information security.

66 "Good care," the primary supervisory area of the IGJ (Article 24 Wkkgz).

67 Explanatory Memorandum p. 25.

68 Article 51, first paragraph, GDPR and Article 6, third paragraph UAVG.

69 See Cooperation Protocol AP-IGJ (Stcrt. 2018, 7023).

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Publication of the advice

This advice will be published on the AP's website within two weeks.

Yours sincerely,
Authority Personal Data,

Katja Mur
Board Member